



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : فاطمه مخلوف عبداللطيف يحي
المحافظة : بني سويف الحي / المدينة : بني سويف
رقم البطاقة : 27101290102807 رقم التليفون : 01123208680
العنوان : منيه الجيد
العمر (تاريخ الميلاد) : 29-01-1971 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمة مخلوف عبداللطيف يحيى
National ID: 27101290102807



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



الجمهورية العربية السورية
مديرية المصالح العامة

فاطمة

مخلوف عبد اللطيف يحيى

قرية منية الجيد

مركز بيا - بتي سوف

١٩٧١/٠١/٢٩

LV3922774

٢٧١.٠١٢٩.٠١.٢٨.٧

١١١٨٩٨٢٨٤٢

٠١١٢٢٢-٨٦٨٠

عش ٧/٨



٢٠٢٢/٠٦

٢٧١.٠١٢٩.٠١.٢٨.٧

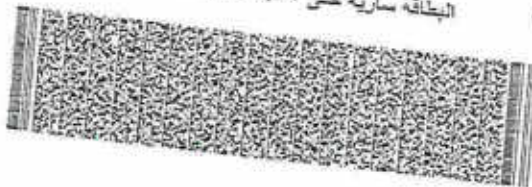
حاصلة على بطون تجارية

متروجة

مسلمة

أشرف عمر عبد اللطيف يحيى

البطاقة سارية حتى ٢٠٢٣/٠٦/٢٩



Patient's Name: Fatima Makhlouf Abdallah
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

CT CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE:

Contrast-enhanced CT examination of the chest, abdomen, and pelvis was performed following intravenous administration of iodinated contrast material. Multiplanar reformatted images were obtained.

FINDINGS:

CHEST:

The visualized portion of the left breast shows an irregular, heterogeneous soft-tissue mass. The lesion measures 4.0 x 2.9 cm in cross-sectional anatomy. Further dedicated breast imaging and tissue diagnosis are recommended.

Multiple enlarged bilateral axillary lymph nodes are identified, more prominent on the left, demonstrating suspicious morphology. The largest left axillary lymph node measures 2.3 x 2.0 cm. The largest right axillary lymph node measures 1.7 x 1.1 cm.

No definite enlarged mediastinal or hilar lymph nodes are identified.

Bilateral lower lobe ground-glass opacities with no focal pulmonary mass or suspicious pulmonary nodule.

No pleural effusion or pneumothorax.

The heart and great vessels are unremarkable on this examination.

No destructive osseous lesion is identified in the visualized thorax.

ABDOMEN AND PELVIS:

The liver is normal in size and enhancement, with no definite focal suspicious hepatic lesion.

The gallbladder and biliary tree are unremarkable.

The spleen, pancreas, and adrenal glands show no focal suspicious abnormality.

Both kidneys enhance symmetrically, with no hydronephrosis or suspicious focal renal lesion.

No significant retroperitoneal or pelvic lymphadenopathy is identified.

No ascites or peritoneal nodularity is seen.

The bowel loops are not dilated.

The urinary bladder is grossly unremarkable.

No focal suspicious pelvic organ abnormality is identified on CT.

Small para-umbilical hernia containing bowel loops.

No definite suspicious lytic or sclerotic osseous lesion is identified within the visualized skeleton.

IMPRESSION:

Patient's Name: Fatima Makhoulf Abdallah
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

1. Irregular left breast soft-tissue mass, suspicious for primary breast malignancy. Dedicated diagnostic mammography/targeted breast ultrasound and histopathological correlation are recommended.
2. Bilateral suspicious axillary lymphadenopathy, more pronounced on the left, concerning for nodal metastatic involvement. Ultrasound-guided assessment/biopsy may be considered.
3. Bilateral lower lobe ground-glass opacities (? Post inflammatory sequel).

Ibrahim

Ibrahim

Patient ID: 172866	Patient Name English: Fatamh mkhlof
Age	2026/07/05 11:11:18 AM :Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography and high-resolution ultrasound
Examination.

Clinical summary: 54 years old female patient with left breast lump.

Detailed Findings

Left breast:

- Scattered fibro glandular densities (ACR density B).
- Left breast shows ill-defined middle 1/3 UIQ spiculated dense mass lesion with architectural distortion and about 5 m from the nipple associated segmental linear branching microcalcifications superior and medial to previously mentioned lesion.
- Complementary US revealed left irregular spiculated hypoechoic mass 11 o'clock position and measuring about 16 x16 mm and about 5 cm from nipple and 5 mm from the skin. Associated focal ill-defined area of altered parenchyma medial to the lesion located at 11:00-12:00 clock position.
- Normal nipple/areolar complex.
- Multiple enlarged axillary lymph nodes at level I, II & III, some of them shows effaced fatty hilum, largest one at level I measures about 20 x 8 mm with focal cortical thickening measuring about 5 mm.
- Few enlarged supraclavicular lymph nodes, largest one measures about 5 x 4 mm.

Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** shows no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Left breast (BIRADS 5):** left UIQ malignant process with ipsilateral pathological axillary and supraclavicular lymph nodes as described. For tru cut biopsy and CESM.
- **Right breast (BIRADS 1):** normal study.

Much obliged,
Dr. Hedaya Nour, MD
Dr Alaa Ahmed, MSC

Name : Fatima Makhlouf Abdul Latif Yehia

Date: 18-09-2026 06:10

Branch : Dokki

Medical No. : 464287

Ref.Physician Prof: Goustav Roussev Egypt

تعاقد مستشفيات : Ins.Company:

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 55-year-old female with left breast carcinoma. Presents with generalized bony aches. Referred for baseline skeletal staging.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- A discrete focal area of increased tracer uptake is identified at the right sternoclavicular articulation/upper sternal region.
- The remaining rib cage and left sternal border show symmetric physiological activity.
- Mild-to-moderate non-specific low-grade heterogeneous uptake is noted along the thoracolumbar spine, consistent with spondylodegenerative disease.
- Pelvic bones show normal symmetric distribution.
- Bilateral symmetric low-grade uptake is noted at the knee joints, consistent with degenerative arthropathy.
- The remainder of the skeleton shows normal physiological distribution.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Focal uptake at the right sternoclavicular articulation—differential includes benign degenerative/arthritis change versus an early focal osseous deposit; targeted CT is recommended for correlation.
- No scintigraphic pattern of diffuse osteoblastic skeletal metastases.

Dr. Alaa Nagi

MD Nuclear Medicine

Name : Fatima Makhlouf Abdul Latif Yehia

Date : 18-09-2026 06:10

Branch : Dokki

Medical No. : 464287

Ref.Physician Prof : Goustav Roussey Egypt

تعلق مستشفى : Ins.Company :

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Dr. Alaa Nagi

MD Nuclear Medicine

Patient Name	فاطمة مختار عبد اللطيف	Case No.	341070
Sex/Age	Female 55 Y	Patient No.	24593
Registered Date	09/7/2026 10:29:12 AM	Referred By Dr.	Out Patient
Validation Date	05/8/2026 09:40:01 AM	Branch	Gustave Roussy
Reporting Date	05/8/2026 01:35:13 PM	Pathology No.	2160/26

invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



مركز فاطمة مختار عبد اللطيف (987) - مبنى (الطريق) - طابق (4) - مبنى (4) - First Floor-Building No. 987- Nile Corridor - Mistr El Qadema - Cairo, Egypt

Patient Name	فاطمة مخلوف عبداللطيف	Case No.	341070
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Reporting Date	05/8/2026 01:35:13 PM	Pathology No.	2160/26

- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.
- Minimal low-grade ductal carcinoma in situ (Cribriform pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).
- Her2: Strong (score 3+).
- Ki67%: 25%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the



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Patient's Name:

Fatima Makhlouf Abdallah

Date:

2026-08-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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3. Bilateral lower lobe ground-glass opacities (? Post inflammatory sequel).

Ibrahim

Ibrahim



Patient's Name:

Kafura Matchloui Abdallah

Date:

2026-08-24

Referred by:

Prof Dr. no doctor

Hospital:

Goustave Roussay International Egypt

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Foruhin

Foruhin



Patient's Name:

Fatima Makhlof Abdallah

Date:

2026-08-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

CT CHEST, ABDOMEN AND PELVIS WITH CONTRAST

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No definite enlarged mediastinal or hilar lymph nodes are identified. Bilateral lower lobe ground-glass opacities with no focal pulmonary mass or suspicious pulmonary nodule.

No pleural effusion or pneumothorax.

The heart and great vessels are unremarkable on this examination.

No destructive osseous lesion is identified in the visualized thorax.

ABDOMEN AND PELVIS:

The liver is normal in size and enhancement, with no definite focal suspicious hepatic lesion. The gallbladder and biliary tree are unremarkable.

The spleen, pancreas, and adrenal glands show no focal suspicious abnormality.

Both kidneys enhance symmetrically, with no hydronephrosis or suspicious focal renal lesion.

No significant retroperitoneal or pelvic lymphadenopathy is identified.

No ascites or peritoneal nodularity is seen.

The bowel loops are not dilated.

The urinary bladder is grossly unremarkable.

No focal suspicious pelvic organ abnormality is identified on CT.

Small para-umbilical hernia containing bowel loops.

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IMPRESSION:

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Sex/Age	Female 55 Y	Patient No.	24593
Registered Date	09/7/2026 10:29:12 AM	Referred By Dr.	Out Patient
Validation Date	05/8/2026 09:40:01 AM	Branch	Gustave Roussy
Reporting Date	05/8/2026 01:35:13 PM	Pathology No.	2160/26

Clinical Data: Left breast UIQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 09/07/2026.

Gross Examination:

- Four grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Minimal low-grade DCIS (Cribriform pattern).
- **Angioinvasion:** Absent
- **Perineural invasion:** Absent

Results:

*** IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 2% of tumor cells (score 3/8).



جوستاف رويسى الدولى - مصر - علم الأمراض
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nasr El Gharbia - Cairo, Egypt

Patient ID: 172866	Patient Name English: Fatamh mkhlof
Age	2026/07/05 11:11:18 AM Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound

Examination.

Clinical summary: 54 years old female patient with left breast lump.

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- Complementary US revealed left irregular spiculated hypoechoic mass 11 o'clock position and measuring about 16 x16 mm and about 5 cm from nipple and 5 mm from the skin. Associated focal ill-defined area of altered parenchyma medial to the lesion located at 11:00-12:00 clock position.
- Normal nipple/areolar complex.
- Multiple enlarged axillary lymph nodes at level I, II & III, some of them shows effaced fatty hilum, largest one at level I measures about 20 x 8 mm with focal cortical thickening measuring about 5 mm.
- Few enlarged supraclavicular lymph nodes, largest one measures about 5 x 4 mm.

Right breast:

- Scattered fibro glandular densities (ACR density B).
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Impression:

- **Left breast (BIRADS 5):** left UIQ malignant process with ipsilateral pathological axillary and supraclavicular lymph nodes as described. For tru cut biopsy and CESM.
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*Much obliged,
Dr. Hedaya Nour, MD
Dr Alaa Ahmed, MSC*



Patient's Name: Fatima Makhoul Abdallah
Date: 2026-08-24
Referred by: Prof. Dr. no doctor
Hospital: Goustave Roussey International Egypt

CT CHEST, ABDOMEN AND PELVIS WITH CONTRAST

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No definite enlarged mediastinal or hilar lymph nodes are identified.
Bilateral lower lobe ground-glass opacities with no focal pulmonary mass or suspicious pulmonary nodule.

No pleural effusion or pneumothorax.

The heart and great vessels are unremarkable on this examination.

No destructive osseous lesion is identified in the visualized thorax.

ABDOMEN AND PELVIS:

The liver is normal in size and enhancement, with no definite focal suspicious hepatic lesion. The gallbladder and biliary trees are unremarkable.

The spleen, pancreas, and adrenal glands show no focal suspicious abnormality.

Both kidneys enhance symmetrically, with no hydronephrosis or suspicious focal renal lesion.

No significant retroperitoneal or pelvic lymphadenopathy is identified.

No ascites or peritoneal nodularity is seen.

The bowel loops are not dilated.

The urinary bladder is grossly unremarkable.

No focal suspicious pelvic organ abnormality is identified on CT.

Small para-umbilical hernia containing bowel loops.

No definite suspicious lytic or sclerotic osseous lesion is identified within the visualized skeleton.

IMPRESSION:

جمهورية مصر العربية
بطاقة تحفيق الشعبية



قاطمه
مخلوف عبداللطيف يحي
قرية منية الجيد
مركز ببا - بني سويف

٢٧١/٠١
٢٧١.٠١٢٩.٠١.٢٨.٧

LV3922774



٢٠٢٦/٠٦ ٢٧٨٠١٢٨٠١٠٢٨٠٧

حاصله على دبلوم تجارة

انثى مسلمة متزوجة
على عمر عبد القليل يحيى

البطاقة سارية حتى ٢٠٣٣/٠٦/٢٩



9/6/26, 10:01 AM

code:09 – Name_A MDT FORM – Name_E MDT FORM

جولف
روني
١٩٩٩

٩-٦



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية على اطلاع
١٣٤٦

09/06/2026

□

MDT FORM

Name:	فهد مفرح عبدالله ربي	MRN:	13087
Age:	56 Y	ID:	27101290102807
Menopausal Sta	post menopausal	Phone:*	0118983843
NRS SCORE:	0	Marital Status:	married
BMI:	0	Family History:	- ve
Gender		Reason of MDT:	new
CO Morbidity:	free	ECOG Performance Status	0 1 2 3 4
Surgical History:*	appendectomy		

Clinical Data:

R

L



T N M

Sonomammography:

LT UIQ MALIGNANT PROCESS WITH IPSILATERAL PATHOLOGICAL AND
SUPRA CLAVICULAR LYMPH NODES
Biomarker/Molecular:

Q1 Right Side Q2 Q3 Q4 Q1 Left Side Q2 Q3 Q4

Metis Workup:

CAP BILATERAL SUSPICIOUS AXILLARY LYMPH NODES
BONE SCAN REQUESTED
TRU CUT BIOPSY FROM CONTRA LATERAL AXILLA REQUESTED

Clinical Staging:

Pathological Staging:

MDT Outcome & Plan

ct not attached and no bx from contralat axillary LN bx mbot attached to
contact the patient to bring them ASAP

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

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دولة الكويت
الجمهورية
وزارة الصحة



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للحساسية والمناعة
بمصر

09/27/2026

MDT FORM

Name:	نظم مطر صالح بن	MRN:	13087
Age:	56 Y	ID:	27101290102807
Menopausal Sta	post	Phone:*	01123208680
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve
Gender		Reason Of MDT:	
CO Morbidity:	free	new case	
Surgical History:*	appendectomy	ECOG Performance Status	
		☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4	

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 3/8 **HER2:** +3 **KI67:** 25 %

History:

Sonomammography:

LT UIQ MALIGNANT PROCESS WITH IPSILATERAL PATHOLOGICAL AND SUPRA CLAVICULAR LYMPH NODES bil 5m (5-7-2026) : Left breast shows ill-defined middle 1/3 UIQ spiculated dense mass lesion with architectural distortion and about 5 m from the nipple associated segmental linear branching microcalcifications superior and media to previously mentioned lesion.

- hypoechoic mass 11 o'clock position and measuring about 16 x16 mm and about 5 cm.
- Multiple enlarged axillary lymph nodes at level I, II & II, some of them shows effaced fatty hilum, largest one at level I measures about 20 x 8 mm with focal cortical thickening measuring about 5 mm. Few enlarged supraclavicular lymph nodes, largest one measures about 5*4 mm CT4d N3 Mx (about to fungate)

Biomarker/Molecular

Metastatic Workup:

CAP (24-8-2026) :

-BILATERAL SUSPICIOUS AXILLARY LYMPH NODES no pulmonary , hepatic , osseous lesions bone scan (18-9-2026) :
-focal uptake at Rt sternoclavicular articulation arthritic VS focal osseous deposits for Ct correlation
Lt breast mass rapidly progressing clinically Ct4d about to fungate bx from contralateral axillary lns still pending (pt afraid to take biopsy)
pt in severe pain

Recommendations:

MDT Outcome & Plan

BX OF CONTRALATERAL AXILLA
MEANWHILE START TC
PORTA CATH/ECHO
REMDT (ADD HERCEPTIN IV VS HP)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كوكبي	د. محمد الطراوي	د. رانيا رمضان	

about:blank

Name : Fatima Makhlouf Abdul Latif Yehia

Date: 18-09-2026 06:10

Branch : Dokki

Medical No. : 464287

Ref.Physician Prof: Goustav Roussev Egypt

تعاقد مستشفيات : Ins.Company:

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 55-year-old female with left breast carcinoma. Presents with generalized bony aches. Referred for baseline skeletal staging.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- A discrete focal area of increased tracer uptake is identified at the right sternoclavicular articulation/upper sternal region.
- The remaining rib cage and left sternal border show symmetric physiological activity.
- Mild-to-moderate non-specific low-grade heterogeneous uptake is noted along the thoracolumbar spine, consistent with spondylodegenerative disease.
- Pelvic bones show normal symmetric distribution.
- Bilateral symmetric low-grade uptake is noted at the knee joints, consistent with degenerative arthropathy.
- The remainder of the skeleton shows normal physiological distribution.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Focal uptake at the right sternoclavicular articulation—differential includes benign degenerative/arthritis change versus an early focal osseous deposit; targeted CT is recommended for correlation.
- No scintigraphic pattern of diffuse osteoblastic skeletal metastases.

Dr. Alaa Nagi

MD Nuclear Medicine

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Patient Name	فاطمة مختار عبد اللطيف	Case No.	341070
Sex/Age	Female 55 Y	Patient No.	24593
Registered Date	09/7/2026 10:29:12 AM	Referred By Dr.	Out Patient
Validation Date	05/8/2026 09:40:01 AM	Branch	Gustave Roussy
Reporting Date	05/8/2026 01:35:13 PM	Pathology No.	2160/26

invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



مركز فاطمة مختار عبد اللطيف (987) - مسعى المسعى - مبنى (4) - First Floor-Building No. 987- Nile Corridor - Most El Qadema - Cairo, Egypt

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Patient's Name:

Fatima Makhlouf Abdallah

Date:

2026-08-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

1. Irregular left breast soft-tissue mass, suspicious for primary breast malignancy. Dedicated diagnostic mammography/targeted breast ultrasound and histopathological correlation are recommended.
2. Bilateral suspicious axillary lymphadenopathy, more pronounced on the left, concerning for nodal metastatic involvement. Ultrasound-guided assessment/biopsy may be considered.
3. Bilateral lower lobe ground-glass opacities (? Post inflammatory sequel).

Ibrahim

Ibrahim



Patient's Name:

Kafura Matchloui Abdallah

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2026-08-24

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Prof Dr. no doctor

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Fatima Makhlof Abdallah

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Hospital:

Goustave Roussey International Egypt

CT CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE:

Contrast-enhanced CT examination of the chest, abdomen, and pelvis was performed following intravenous administration of iodinated contrast material. Multiphase reformatted images were obtained.

FINDINGS:

CHEST:

The visualized portion of the left breast shows an irregular, heterogeneous soft-tissue mass. The lesion measures 4.0 x 2.9 cm in cross-sectional anatomy. Further dedicated breast imaging and tissue diagnosis are recommended.

Multiple enlarged bilateral axillary lymph nodes are identified, more prominent on the left, demonstrating suspicious morphology. The largest left axillary lymph node measures 2.3 x 2.0 cm. The largest right axillary lymph node measures 1.7 x 1.1 cm.

No definite enlarged mediastinal or hilar lymph nodes are identified. Bilateral lower lobe ground-glass opacities with no focal pulmonary mass or suspicious pulmonary nodule.

No pleural effusion or pneumothorax.

The heart and great vessels are unremarkable on this examination.

No destructive osseous lesion is identified in the visualized thorax.

ABDOMEN AND PELVIS:

The liver is normal in size and enhancement, with no definite focal suspicious hepatic lesion. The gallbladder and biliary tree are unremarkable.

The spleen, pancreas, and adrenal glands show no focal suspicious abnormality.

Both kidneys enhance symmetrically, with no hydronephrosis or suspicious focal renal lesion.

No significant retroperitoneal or pelvic lymphadenopathy is identified.

No ascites or peritoneal nodularity is seen.

The bowel loops are not dilated.

The urinary bladder is grossly unremarkable.

No focal suspicious pelvic organ abnormality is identified on CT.

Small para-umbilical hernia containing bowel loops.

No definite suspicious lytic or sclerotic osseous lesion is identified within the visualized skeleton.

IMPRESSION:

Medical No. : 8810

Patient's Name: Fatima Makhlouf Abdallah

Date: 2026-09-27

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Duplex Examination of the venous system of left upper limb revealed: -

- The subclavian, axillary, radial & ulnar veins of the examined upper limb all are patent, totally compressible with normal color filling & preserved respiratory variation.
- **No evidence of D.V.T.**
- **Patent cephalic & basilic veins along their course, no evidence of superficial thrombophlebitis at time of scan**

OPINION:-

No evidence of DVT or superficial thrombophlebitis at time of scan

Moderate interstitial edema & multiple areas of increase subcutaneous fat echogenicity likely inflammatory condition (cellulitis) for clinical correlation.

DR: Nermeen Ramadan.MD

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Clinical Data: Left breast UIQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 09/07/2026.

Gross Examination:

- Four grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Minimal low-grade DCIS (Cribriform pattern).
- **Angioinvasion:** Absent
- **Perineural invasion:** Absent

Results:

*** IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 2% of tumor cells (score 3/8).



مركز جوستاف رويسى الدولى - مصر - علم الأمراض
 Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Nasr El Gharbia - Cairo, Egypt

Patient ID: 172866	Patient Name English: Fatamh mkhlof
Age	2026/07/05 11:11:18 AM Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound

Examination.

Clinical summary: 54 years old female patient with left breast lump.

Detailed Findings

Left breast:

- Scattered fibro glandular densities (ACR density B).
- Left breast shows ill-defined middle 1/3 UIQ spiculated dense mass lesion with architectural distortion and about 5 m from the nipple associated segmental linear branching microcalcifications superior and medial to previously mentioned lesion.
- Complementary US revealed left irregular spiculated hypoechoic mass 11 o'clock position and measuring about 16 x16 mm and about 5 cm from nipple and 5 mm from the skin. Associated focal ill-defined area of altered parenchyma medial to the lesion located at 11:00-12:00 clock position.
- Normal nipple/areolar complex.
- Multiple enlarged axillary lymph nodes at level I, II & III, some of them shows effaced fatty hilum, largest one at level I measures about 20 x 8 mm with focal cortical thickening measuring about 5 mm.
- Few enlarged supraclavicular lymph nodes, largest one measures about 5 x 4 mm.

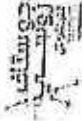
Right breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** shows no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Left breast (BIRADS 5):** left UIQ malignant process with ipsilateral pathological axillary and supraclavicular lymph nodes as described. For tru cut biopsy and CESM.
- **Right breast (BIRADS 1):** normal study.

*Much obliged,
Dr. Hedaya Nour, MD
Dr Alaa Ahmed, MSC*



Patient's Name: Fatima Makhoul Abdallah
Date: 2026-08-24
Referred by: Prof. Dr. no doctor
Hospital: Goustave Roussey International Egypt

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جمهورية مصر العربية
بطاقة تحفيظ الشعبة



قاطمه
مخلوف عبداللطيف يحي
قرية متية الجيد
مركز بيا - بني سويف

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٢٧١.٠١٢٩.٠١.٢٨.٧

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حاصله على دبلوم تجارة



انثى مسلمة متزوجة
على عمر عبد القليل يحيى
البطاقة سارية حتى ٢٠٣٣/٠٦/٢٩

